

137 million ED visits: what the 2015 national sample shows hospital managers

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21,061

sampled visits from 248 EDs; each row is one visit, not one patient

136.9M

U.S. ED visits once weighted by PATWT, matching the NCHS-published total

-9 / -8 / -7

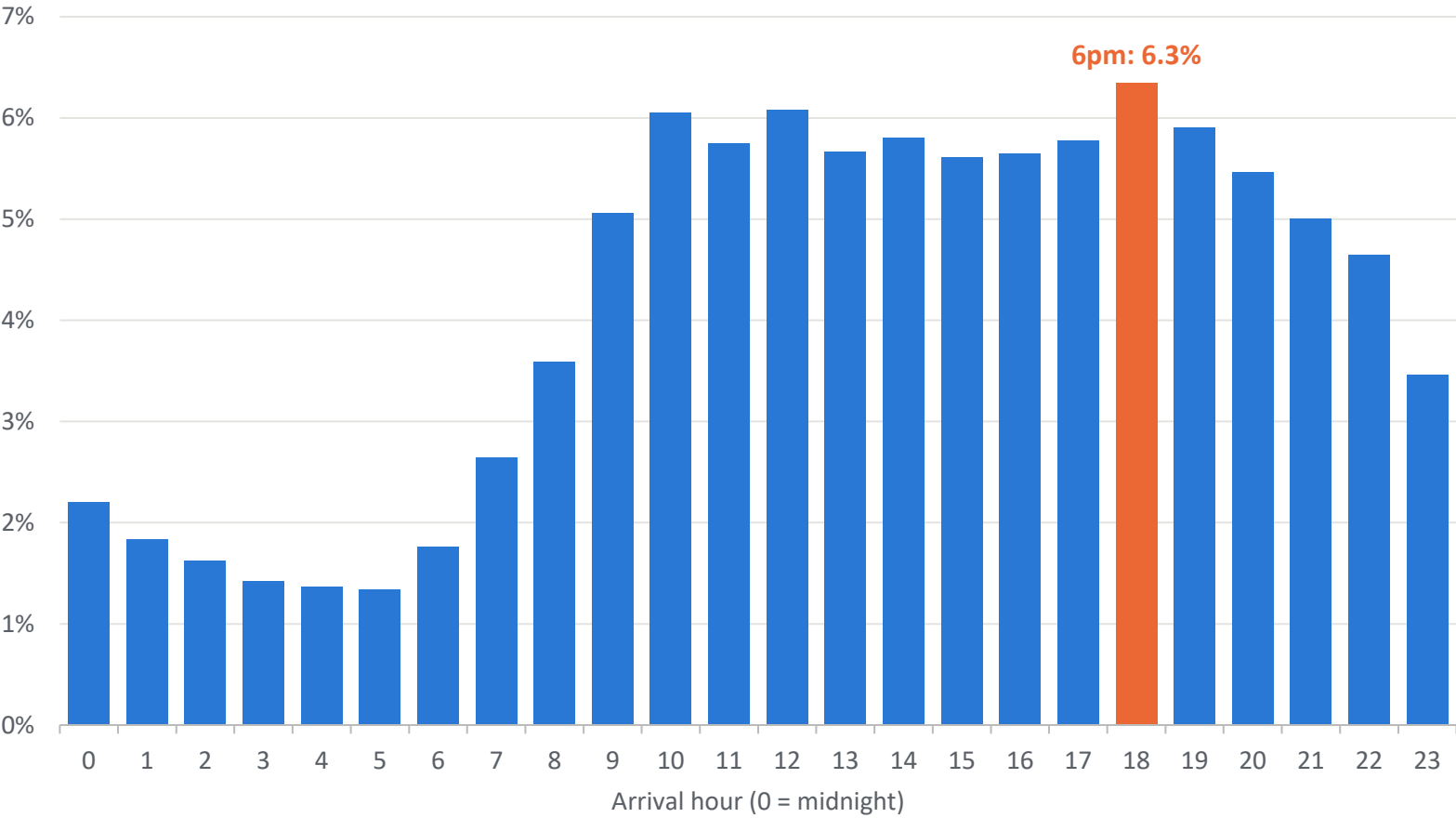
codes NCHS uses for blank, unknown and not-applicable answers; recoded to missing

Method

National totals, shares and rates weighted by PATWT; sample counts shown unweighted. Missing-value codes recoded before any statistic. Waits and stays summarized mainly with medians; the write-up also reports raw means and means capped at the 99.5th percentile. Key estimates compared with figures NCHS publishes: all agree to rounding except wait-time missingness (15.2% vs 15.7%).

Arrivals stay near peak from 10am to 8pm, and the median visit lasts 2.6 hours

Weighted share of daily arrivals by hour (visits with a recorded arrival time, 98.7% of records)



154 min

median length of visit (raw mean 214 min; 210 with values capped at the 99.5th percentile)

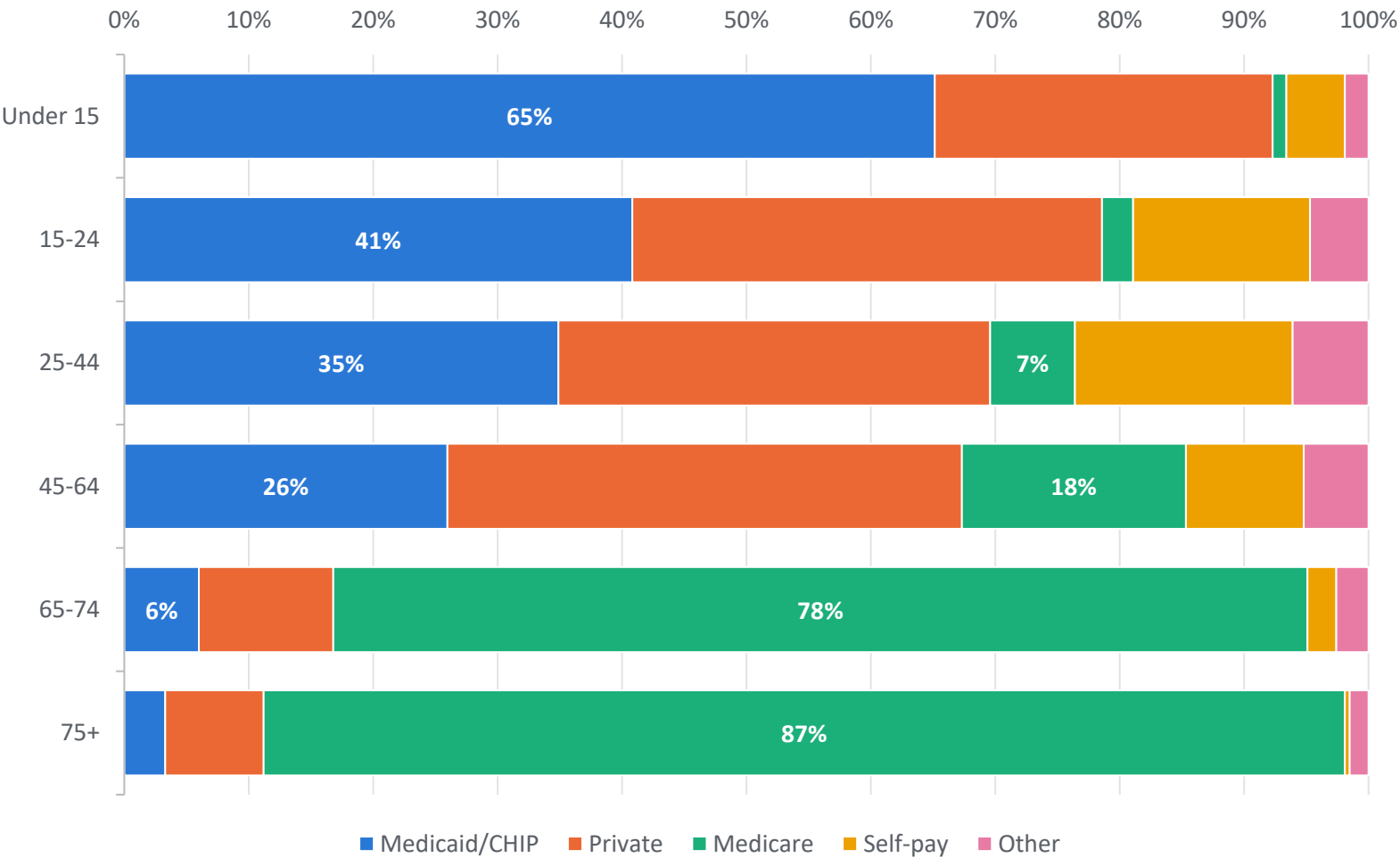
+25%

visits per day on Mondays vs Sundays (432,000 vs 346,000)

Because patients stay about 2.6 hours, the number in the department likely peaks after arrivals do. Staffing plans should weigh time in the department, not arrivals alone.

Medicaid is the most common expected payer, and the expected payer varies sharply with age

Primary expected payer by age group, weighted % of visits with a known expected payer (89.2% of visits)



48.9%

of all visits list Medicaid or Medicare as primary expected payer (54.8% of visits with a known payer)

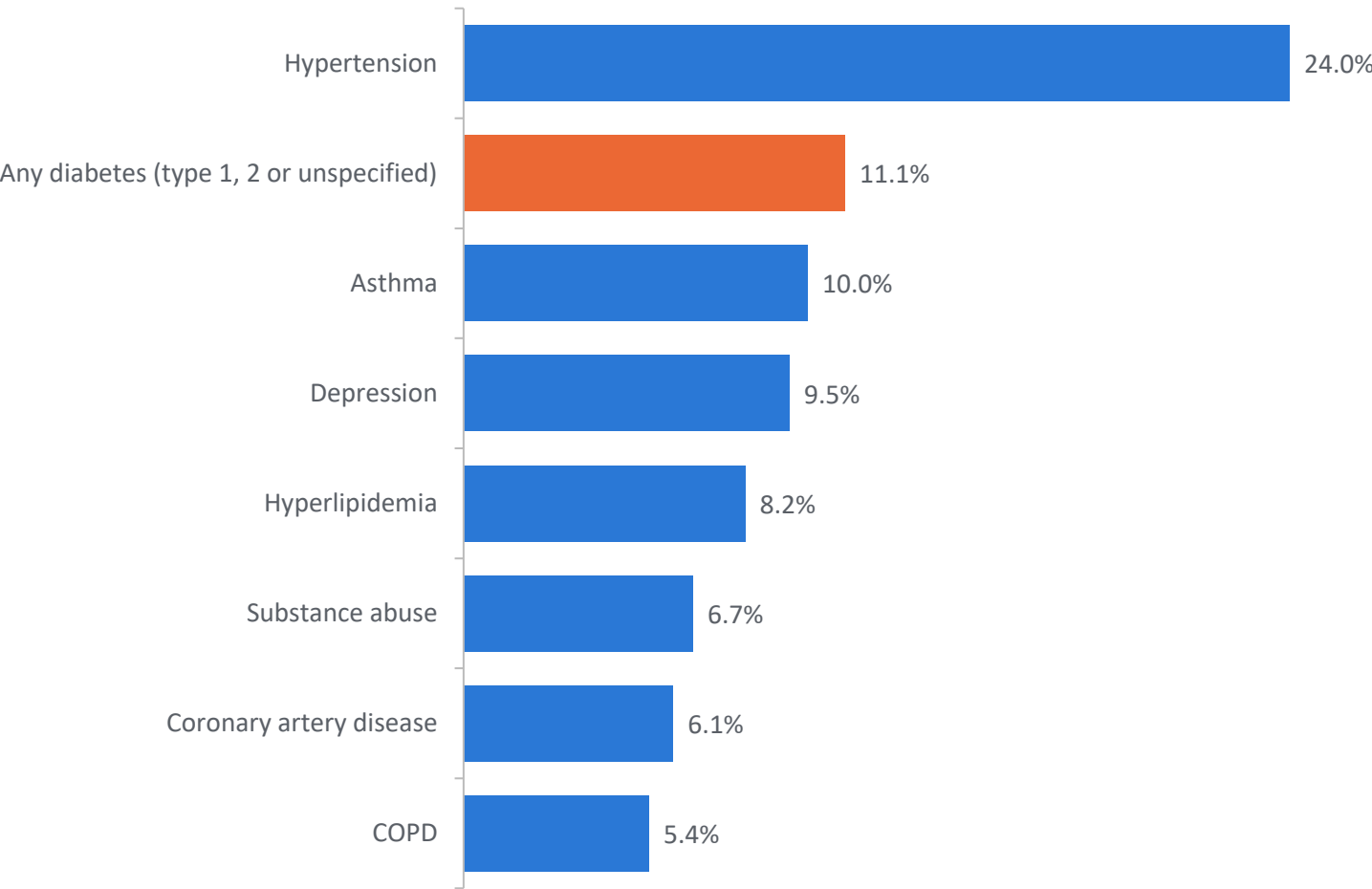
11.0%

self-pay or no charge (visits with a known payer)

Expected payer is not revenue. If public programs pay less per visit than private insurers, this mix limits per-visit revenue; the actual effect depends on contracts and collections.

Nearly half of visits involve a chronic condition, and medication use rises with it

Weighted % of visits with the chronic-condition section completed (98.4% of records)



47.6%

of visits with the section completed record at least one chronic condition (90% at ages 75+)

2.0 → 3.8

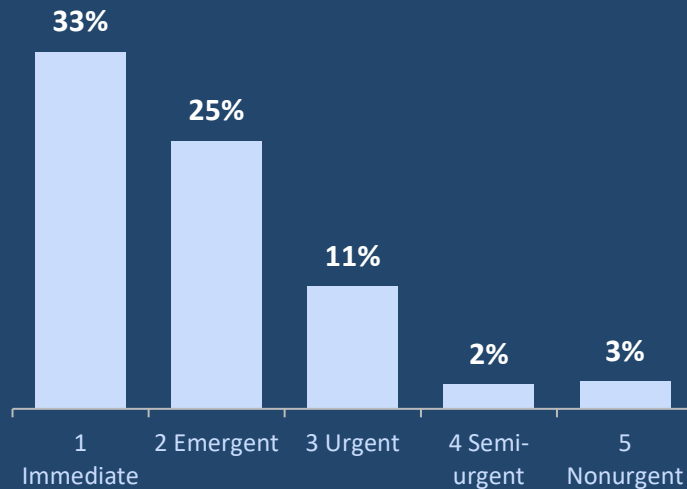
mean medications per visit: no recorded chronic condition vs three or more

Diabetes is coded in three checkboxes: type 2 alone is 4.7%, but any diabetes (highlighted) is 11.1%, second only to hypertension.

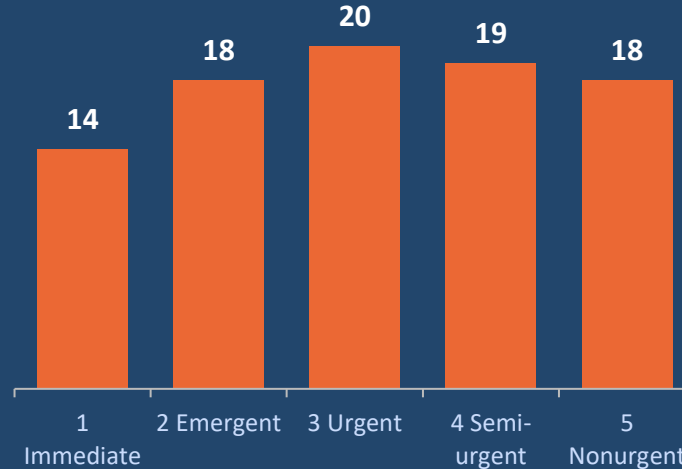
Triage level tracks admission closely; recorded waits differ little beyond level 1

Weighted, visits with triage levels 1-5 (1 = immediate, 5 = nonurgent)

Admitted to hospital (%)



Median recorded wait to see a provider (min)



Reading these results

Descriptive, not causal: case mix and hospital differences are not controlled.

Wait time is blank for 15.2% of sampled visits and not applicable (not seen by a provider) for 3.4%, so 18.6% have no usable wait; missingness is higher where triage was not recorded and in the Northeast and West.

Weighted point estimates without survey-design standard errors.

A question for hospital managers

Emergent (level 2) and nonurgent (level 5) patients have the same recorded median wait, 18 minutes. Do local timestamp data show the same pattern, and if so, where does the delay occur?